

Session Objectives

THE STUDENT PHYSICIANS WILL BE ABLE TO:

1. **Demonstrate** the components of the **history and physical** examination of the patient with a behavioral or mental health complaint
2. **Evaluate** the patient utilizing **history taking, observational skills, active listening, facilitated responses in addition to the physical exam**
3. **Distinguish normal versus abnormal encounter** findings-use of screening measures
4. **Anticipate** and discuss common **abnormal** encounter findings
5. Recall the appropriate way to **document** common normal and abnormal findings of history and examination intake by **writing a SOAP note within 9 minutes**.

MAIN POINTS:

- History considerations for the patient with a behavioral or mental health complaint
- Physical exam
- Osteopathic Treatments
- SOAP note writing



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Epidemiology & Impact

The Evaluation of an Adult Patient with a Behavioral Complaint

During the interview

- Maintain professionalism at all times
 - Be aware of pre-existing biases in yourself-non-judgmental
- Use the patient's own language
 - Repeating their words in your statements can help show the patient you are listening and trying to understand.
- Use appropriate **transition statements** when you move onto topics that are sensitive or personal
- Educate/counsel patient **after** you have gathered the history & completed the physical exam

Observation of the patient

How do the **patient's actions/appearance** contribute to the information they are giving you?

While interviewing the patient, you will concomitantly observe the patient's

- Appearance
 - Speech patterns-pitch, tone
 - Behavior
 - Attitude
 - Mental status
 - Posture
 - Mood
 - Affect
- Think back to the Cultural Competency and General appearance DPR lectures

Chief complaint & current history

- Chief complaint
 - In the patient's own words (always)
- HPI
 - Use open ended questions followed by closed ended questions for clarification
 - Obtain a thorough account and timeline of the patient's symptoms/concern
 - Patient Health Questionnaire-2 (PHQ-2) - Mental Disorders Screening-Depression
 - Generalized Anxiety Disorder Screening-2 (GAD-2)

Current history

- When did symptoms begin?
 - Acute versus gradual changes
 - What triggered the symptoms?
- How have the symptoms changed over time?
- Can they describe what they are feeling or experiencing?
- Anything make it better? Anything make it worse?
- How have they impacted them on daily basis at home, at work, at school, etc.?
- How have they been coping?

Current history

- **Assess suicide/homicide risk**
 - Have they had thoughts about hurting themselves or other?
 - Have they made any attempts at suicide?
 - If yes, do they have a specific plan or timeline? Do they have access to weapons or the items in their plan?

Suicide Questions

- What method of self-harm was involved?
- How were they discovered?
- How did they feel when help arrived?
- Do they still feel suicidal?

Self-harm

- The patient may be harming themselves in other ways
- Skin picking/scratching
- Hair pulling-BFRBs
 - Cutting
 - Burning
 - Abusing drugs or alcohol

Medical history

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- Past medical history
- Past surgical history
- Medications
- Allergies

Living situation

- Where do they live?
- Who do they live with?
 - Consider that if they have children, the children may be neglected or witnessing the patient's struggle & may need intervention
- Are they in a relationship?
 - What type of relationship? How is that relationship now?

Occupation

- What job does the patient have?
 - How are the relationships at work?
 - How are they performing at work?
- If they are not working, how are they coping financially? Are they in debt?
- Have they served in the military?
 - If so, what was their experience?

CAGE Questionnaire for Detecting Alcoholism

📄 Share

Questions

Points

C : Have you ever felt you should Cut down on your drinking?

☐ Yes +1

☐ No +0

A : Have people Annoyed you to by criticizing your drinking?

☐ Yes +1

☐ No +0

G : Have you ever felt Guilty about your drinking?

☐ Yes +1

☐ No +0

E : Have you ever had a drink first thing in the morning (Eye opener)?

☐ Yes +1

☐ No +0

CAGE score is obtained by adding the points (total points)

0 points

Social history

*Current or past drug use

- Do they use recreational drugs?
- What drugs do they use?-Gateway Drugs-marijuana
- How often do they use the drug(s)?
- How much do they use of the drug(s)?

Social history

- Current or past tobacco use
 - Do they use tobacco products?
 - Which tobacco product?
 - How much do they use on a daily basis?

Review of Systems

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- Ask questions about main organ systems to help determine if there is a physiological component or cause to the psychiatric symptoms that the patient is feeling
 - Constitutional
 - Cardiovascular
 - Pulmonary
 - Endocrine
 - Gastrointestinal
 - Neurological

Physical examination

- May be dependent on the differential diagnosis
- Examine major organ systems to look for signs/symptoms of disease that might cause the patient's presenting complaint
 - Cardiovascular
 - Pulmonary
 - Endocrine
 - Gastrointestinal
 - Neurological

Mental status exam

- -Geriatric lecture
- You need to constantly observe as well as listen to the patient, in order to assess them properly
- There are multiple elements that you will pay attention to

Mental Status Exam

*Psychomotor activity

- Increased, reduced, agitated, abnormal movement, etc.
- Mood-
- How the patient say they feel (sustained emotion)
 - Stated mood versus observed mood
 - Overall emotional state (sad, happy, depressed, elated, anxious, irritable, etc.)

Mental status exam

Affect

- Outward expression of patient's inner emotions/experiences
- Full affect (full range of emotions) versus restricted, blunted or flat
- Appropriate versus inappropriate to the patient's thoughts

Mental Status Exam

- Judgement
 - The ability to come to a sensible conclusion on a specific matter when need to choose from multiple options
 - For example: an inebriated patient decides that they are going to drive home
 - Described as poor, fair, or good.
- Insight
 - Does the patient recognize that they are ill? Do they appropriately attribute their symptoms to their illness/impairment of functioning?
 - Described as intact or not-awareness

Differential diagnosis

- Dependent on all the elements of the history and examination
- Have to rule out **physiological disease/disorders** that could mimic or cause the patient's symptoms
 - For example: **hypothyroidism** in patient with depression
 - For example: **arrhythmia** in patient with anxiety-mimics anxiety

Diagnostic tests to consider

- Dependent on the differential diagnosis but typically include:
 - EKG
 - Complete Blood Count (CBC)-infection, anemic, B12 deficient
 - Comprehensive Metabolic Panel (CMP)-electrolytes, liver fx-effects cognition
 - Thyroid studies
 - Lipid panel
 - Urinalysis-UTI-altered mental status in Geriatrics
- These tests also help to establish a **baseline** since medications can have an effect on glucose, lipids, heart rhythms, etc.

Osteopathic Considerations

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- Assess areas of somatic complaints
- Perform an osteopathic structural exam
 - **Consider techniques to normalize parasympathetic and sympathetic-so body can get back to healing itself**
 - **Suboccipital Release**
 - **Rib raising**
- Treat appropriately based off your structural exam findings
 - Sometimes less is more-patient may not want this

Summary Slide

- Mental health disorders have a strong societal and personal impact
- It is important that the history that is gathered from the patient is as thorough as possible and Individualized!
 - And Includes a proper mental status assessment as well as rule out other underlying or chronic conditions